Atezolizumab + carboplatin + etoposide

National Cancer Control Programme (NCCP) 00689 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Appointments and time in the unit

Day 1 order: atezolizumab, carboplatin, then etoposide. Atezolizumab intravenous (IV) first infusion is 60 minutes, later 30 minutes if tolerated; subcutaneous (SC) injection takes about 7 minutes. Carboplatin infusion is 30 minutes and etoposide IV is 60 minutes. Total unit time also includes checks, preparation and observation. SC atezolizumab does not remove the chemotherapy drips during induction.

STEP 2

Your written formulation plan

Confirm your written route plan. Atezolizumab may be a drip (IV) or an injection under the skin of the thigh (SC). Etoposide may be IV on Days 1–3, or IV on Day 1 with prescribed capsules twice daily on Days 2–3. These schedules use different doses; do not change route or convert doses yourself. A separately prescribed four-week IV atezolizumab maintenance plan uses NCCP 00593 and is outside this 21-day calendar.

STEP 3

Travel, support and medicines

Induction usually involves treatment on three consecutive days or one IV day plus prescribed capsules at home. Ask about transport, work, caring responsibilities and who to call about capsules or symptoms. Carry the immunotherapy alert card.

STEP 4

Tests and maintenance

full blood count (FBC), liver, renal and glucose profile before every cycle; thyroid tests every 3–6 weeks. Scans and other disease assessment follow the consultant plan. Continue symptom review during maintenance and after atezolizumab stops because immune effects may be delayed. Confirm which team books each test and who reviews it before treatment.

Sources: NCCP 00689 v4 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.